

UUID: AE058D5A-7249-44E5-87AA-663B07DA6A63
TCGA-T9-A92H-01A-PR

Redacted



Subject Number:

Site ID:

Please complete ALL information with black ball point pen. Ensure information is legible.
Retain original and email/fax copy to

Sample Collection Date:	Procured by:
Time of Tumor Sample Excision (24 hour clock):	Time Zone:
TUMOR-FROZEN & FRESH SAMPLES FROZEN SAMPLES BATCH SHIPPED (dry ice); FRESH TUMOR SAMPLE, IF COLLECTED, SHIPPED ON DAY OF COLLECTION	
Collect resected tumor in accordance with standard procedures. Weigh the sample and section off ~100mg. Frozen Tumor (Required) Place this sample in the cryovial labeled Frozen Tumor sample and record the weight. Place the cryovial in a can and then into the vapor phase dewar. Record the time placed in dewar.	
Weight of Tissue: <u>270</u> mg (tissue weight must be ≥ 100mg)	Time placed in dewar: (24 hour clock) H H : M M
Fresh Tumor (Optional) ☐ N/A Locate the "Fresh Tumor" label in the kit and affix to the 15ml tube (sent separately). Place any remaining tumor tissue into the 15ml conical tube containing	
Weight of Tissue: <u>690</u> mg (no min. weight specified)	Time placed in (24 hour clock) H H : M M Lot Number of
WHOLE BLOOD SAMPLE (Required) - BATCHED SHIPPED FROZEN (dry ice)	
Sample Collection Date:	Procured by:
Time of Blood Sample Collection (24 hour clock):	Time Zone:
1-10ml EDTA tube. Fill tube completely and mix by gentle inversion at least 8-10 times. Freeze sample on dry ice or place in vapor phase dewar.	
Time placed in dry ice/dewar: (24 hour clock)	H H : M M
BASIC PATHOLOGICAL INFORMATION	
Clinical Diagnosis: (Ex. Lung Cancer) <u>Colon Cancer</u>	Source of Specimen: (Ex. Right Lung) <u>Sigmoid Colon</u>

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Valid from

Page 1 of 2

ICD O 3

Adenocarcinoma NOS 8140/3
Site: Sigmoid colon C18.9
Q23/26/14

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Tissue/Blood Requisition Form

Affix Label Here
Or Write in Sample ID

Site ID:

Subject Number:

Diagnostic Slides (Required) - SLIDES BATCHED SHIPPED

From the diagnostic FFPE block, cut 3 slides with one section ~ 4-5µm on each slide. Label the frosted tip of the slide in pencil with the sample ID (located on the label at the top of this form). Record the ID number of the diagnostic histology cassette and the number of slides. H&E stain the slides and store in a slide mailer.

Diagnostic Histology Cassette ID

Number of Slides Cut 3

Refer to: Study Manual for detailed specimen processing and shipping instructions. (* denotes required samples)

<u>Specimen</u>	<u>Collection Vessel</u>	<u>Transfer Tube</u>	<u>Shipping Status</u>
Frozen Tumor*	None	Cryovial	Same Day – Dry Ice
Frozen Blood*	1-10ml EDTA	None	Same Day – Dry Ice
Fresh Tumor	None	15ml-	Same Day – Refrigerated
Diagnostic Slides*	Histology Cassette	SuperFrost Slides	5-Slide Mailer—Batch

When FROZEN and FRESH samples are ready for shipment please notify by Email notification:

At this time, please also provide your Sample Manifest.

When the Diagnostic slides are ready for shipment please notify by Email notification:

At this

time, please also provide your Sample Manifest.

DO NOT RETURN EMPTY CONTAINERS TO

White copy Retain for your Records

Yellow copy send to pathology for diagnostic slides

Surgical Pathology Final Report **Temporary Copy**

Collected: [REDACTED]

Final Diagnosis

Superior hemorrhoidal lymph node:

One (1) lymph node negative for neoplasm.

Sigmoid colon, segmental resection:

Adenocarcinoma.

-Histologic grade: Moderately to poorly differentiated.

-Tumor size: 6.3 cm in greatest dimension.

-Tumor extension: Tumor penetrates muscularis propria and extends into pericolic fat.

-Lymph-vascular invasion: Definite lymph-vascular invasion is not identified.

-Perineural invasion: Present, focal.

-Surgical margins:

Proximal and distal margins negative for neoplasm.

Radial margin negative for neoplasm.

Mesenteric vascular margin negative for neoplasm.

-Closest surgical margin: Proximal. Tumor comes to within 5.3 cm.

-Lymph nodes: Thirteen (13) of thirteen (13) lymph nodes negative for neoplasm (0/13).

AJCC pathologic stage: pT3.N0.

STAGE 2A IIA
GRADE 2-3

Clinical Information

Colon cancer.

Gross Description

"A. Superior hemorrhoidal lymph node." Received is a 0.5 cm lymph node with attached fat. "A," all.

"B. Sigmoid colon." Received is an 18.6 cm segment of colon with attached pericolic fat. There is a stitch attached to one end of the specimen which is designated as proximal. The proximal margin will be inked orange. There is a 6.3 x 5.8 cm full circumferential ulcerated mass on the mucosa which is 5.3 cm from the proximal margin. There is black ink on the radial margin adjacent to the mass. The mass grossly appears to extend through the full thickness of the wall of the colon and to involve the adjacent pericolic fat. No other masses or lesions are grossly identified on the remaining mucosa. There is a vascular pedicle identified on the pericolic fat. The mass grossly appears to come within 3.8 cm of the pedicle. The margin of the pedicle will be shaved and submitted. On sectioning the pericolic fat, there are 11 possible lymph nodes ranging from 0.4-0.8 cm. "B1," proximal and distal margins and vascular pedicle margin; "B2-B4," random sections mass; "B5," possible regional lymph node; "B6," possible lymph nodes remaining pericolic fat. [REDACTED]

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 12/9/2013	